

Decision Tree: Preventing Missed First Post-Discharge Psychiatric Follow-Up

START

Patient discharged from inpatient psychiatric care



Decision Point 1

Has the first outpatient follow-up appointment been scheduled and confirmed before discharge?

YES

→ Document appointment details in care record

→ Confirm patient understands:

- appointment date/time
- provider/location
- contact instructions
- escalation contact if unable to attend

→ Assigned Case Manager receives continuity ownership notification

Proceed →

NO

→ Escalate immediately to discharge coordinator / responsible care team before discharge

Rule:

Patient should not exit transition without continuity plan documented unless exception justified

Then reassess:

Has appointment now been secured?

Decision Point 2

Is the patient identified as elevated psychiatric risk?

(criteria: suicide risk / disengagement history / medication instability / recent crisis)

YES

→ Enhanced follow-up pathway

Requirements:

- case manager outreach within 24–48 hours
- proactive risk check
- contingency contact plan
- escalation trigger if unreachable

Proceed →

NO

→ Standard follow-up pathway

Requirement:

case manager outreach within 2 working days

Proceed →

Decision Point 3

Was successful patient contact completed within required timeframe?

YES

→ confirm:

- patient status
- medication continuity
- appointment readiness
- barriers to attendance

Document interaction

Proceed →

NO

→ initiate structured outreach protocol

Case manager:

- call all listed numbers
- contact NOK if appropriate
- attempt alternate channels
- document attempts

Timeframe:

same business day escalation if high risk

Proceed →

Decision Point 4

Was patient reached after structured outreach?

YES

→ reassess risk

→ confirm appointment continuity

→ document resolution

Proceed →

NO

→ escalate to:

- CM Lead
- treating consultant
- crisis response pathway if risk concern

This is critical backup path.

No silent failure allowed.

Decision Point 5

Did patient attend first follow-up appointment?

YES

→ continuity maintained

→ document attendance

→ transition to routine care management

END

NO

→ default detected

Immediate actions:

- case manager notified
- same-day review
- outreach initiated within defined timeframe

Proceed →

Decision Point 6

Was missed appointment due to communication/logistics barrier or elevated clinical concern?

Barrier

(ex:

transport, confusion, scheduling)

→ reschedule urgently

→ reinforce continuity plan

Clinical concern

(ex:

withdrawal, silence, deterioration)

→ risk reassessment

→ consultant notification

→ crisis intervention pathway

Decision Point 7**Was continuity restored?****YES**

→ document resolution

→ continue monitoring

END

NO

→ escalate further:

- psychiatric review
- crisis intervention
- outreach intensification
- welfare check if policy appropriate

END